PMA

Quick Reference of Forgotten Formula PMA IV Protocols

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Alpha Lipoic Acid (ALA) IV Therapy

Section	Details
Dosage	Standard dose: 300–600 mg per infusion Therapeutic range: 300–1,200 mg depending on indication
Dilution	Dilute in 100-250 ml normal saline (0.9% NaCl) Final concentration based on desired dose and tolerance
Infusion	Infuse slowly over 30-90 mins Frequency: 1–2 times weekly, individualized per response
Phases of Treatment	Initial: 300 mg, 1–2× weekly for 4–6 weeks Maintenance: 600 mg weekly Longevity: 300–600 mg monthly for ongoing antioxidant support
Monitoring & Precautions	Monitor: Blood glucose (ALA may lower levels; caution in diabetics or those on hypoglycemics) Contraindicated: Hypersensitivity Caution: When used with insulin or oral hypoglycemic agents
Adjuncts	⚠️ **Administer only after ALA infusion. Do not mix or co-infuse adjunct nutrients (e.g., B-complex, Vitamin C, glutathione) in the same IV bag. Use separate IVs if indicated.** Sequential administration may enhance mitochondrial and detoxification support.
Notes	Protect from light (ALA is photosensitive) Use within 4 hours of dilution Label infusion with dose, date, and patient initials

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DMSO IV Therapy

Section	Details
Dosage	Common range: 25–50 mL DMSO (10–20% solution) Adjust based on patient tolerance and indication
Dilution	Dilute in 250–500 mL normal saline (0.9% NaCl) Typical infusion strength: 10–20%
Infusion	Infuse slowly over 1–2 hours Frequency: 1–2 times weekly, individualized per response
Phases of Treatment	Initial: 25 mL at 10% → titrate upward Maintenance: 25–50 ml at 10–20% weekly/biweekly Longevity: Monthly supportive infusions as indicated
Monitoring & Precautions	Monitor renal & hepatic function Contraindicated: Severe renal/hepatic impairment Expected: Garlic-like odor/taste; counsel patients
Adjuncts	⚠️ **Administer only after DMSO infusion. Do not mix or co-infuse adjunct nutrients (e.g., vitamin C, magnesium, or other supportive nutrients) in the same IV bag.** **U**se separate IVs if indicated.
Notes	Use sterile technique Label infusion with concentration & date Store DMSO at room temperature, away from light

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EDTA IV Therapy

Section	Details
Dosage	Standard range: 1,500–3,000 mg per session Up to 50 mg/kg (maximum 3,000 mg) depending on patient weight and indication
Dilution	Dilute in 500 ml normal saline (0.9% NaCl) 250 ml may be used for smaller doses or sensitive patients
Infusion	Infuse slowly over 2–3 hours (slow drip) Frequency: 1–2 times weekly, individualized per protocol
Phases of Treatment	Initial: 1–2 infusions weekly for 6–12 weeks Maintenance: Weekly to biweekly for 2–4 months Longevity: Monthly or quarterly, adjusted per laboratory results
Monitoring & Precautions	Labs: BUN, creatinine, electrolytes (especially calcium & magnesium) Contraindications: Severe renal impairment, hypocalcaemia Note: Slow infusion minimizes risk of hypocalcaemia or arrhythmias
Adjuncts	⚠️ **Administer only after EDTA infusion. Do not mix or co-infuse adjunct nutrients in the same IV bag. Use separate IVs if indicated.** Common adjuncts include magnesium, B-complex, vitamin C, or trace minerals to support mineral balance and recovery.
Notes	Maintain adequate hydration before and after infusion Replete essential minerals post-treatment Label infusion with concentration, date, and patient identifier

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Glutathione IV Therapy

Section	Details
Dosage	Common dose: 600–2,000 mg per infusion Higher doses: Up to 4,000 mg used in oxidative stress and detoxification protocols
Dilution	Typically diluted in 50–100 mL normal saline (0.9% NaCl) Often administered as an IV push following a larger infusion (e.g., Vitamin C or Myers')
Infusion	Slow IV push: Over 10–20 minutes Or drip infusion: 50–100 mL saline over 20–30 minutes Frequency: 1–3 times weekly, individualized
Phases of Treatment	Initial: 600–1,200 mg, 2–3× weekly Maintenance: 1,200–2,000 mg weekly Longevity: 1,200–2,000 mg monthly for ongoing antioxidant and detox support
Monitoring & Precautions	Generally, well tolerated Caution: In asthmatic patients (may trigger mild bronchospasm) Monitor: Liver function during long-term use
Adjuncts	⚠️ **Administer after Glutathione or other primary infusions. Do not mix or co-infuse with other nutrients in the same IV bag.** Common adjuncts include Myers' cocktail, NAD^+, or ozone therapy for synergistic antioxidant and cellular support.
Notes	Use preservative-free glutathione Prepare freshly before infusion Label with dose, date, and initials

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Hydrogen Peroxide IV Therapy

Section	Details
Dosage	Standard concentration: 3.7% hydrogen peroxide (diluted before infusion) Final infusion concentration: ~0.03% in IV fluid Starting dose: 0.5 mL of 3.7% in 250 ml saline; titrate upward gradually
Dilution	Dilute in 250–500 ml normal saline (0.9% NaCl) Adjust total volume to achieve a final concentration of ~0.03% hydrogen peroxide
Infusion	Infuse slowly over 60–90 minutes Frequency: 1–2 times weekly, individualized per response
Phases of Treatment	Initial: 0.5 mL of 3.7% in 250 mL saline; assess tolerance Maintenance: 1–2 ml of 3.7% in 250–500 mL saline weekly or biweekly Longevity: Monthly infusions for immune and oxidative support
Monitoring & Precautions	Monitor: Oxygenation, pulmonary status, renal function Contraindications: COPD, emphysema, poor lung capacity, oxidative stress disorders Caution: Use carefully in patients with cardiovascular instability
Adjuncts	⚠️ **Administer after hydrogen peroxide infusion. Do not mix or co-infuse adjuncts in the same IV bag.** Common adjuncts include vitamin C, minerals, antioxidants, or glutathione to support redox balance and recovery.
Notes	Use only medical-grade hydrogen peroxide (3.7%) Strict aseptic preparation required Label infusion clearly with dilution, date, and initials

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Myers' Cocktail IV Therapy

Section	Details FF PMA Myer's per 10ml in 50ml Bottle
Dosage	Per 10 ml contains: Vitamin C 7.5g , Hydroxocobalamin 200mcg, Pyridoxine 250mg , Pantothenate NA 500mg , Mag. Sulfate 5mg , Mag. Chloride 1.6g, Thiamine HCL 150mg, Riboflavin 25mg , Nicotinamide 250mg, Selenium 25mcg Copper 0.5mcg, Chromium 4mcg, Zinc 5mg
Dilution	Usually prepared in 250 mL normal saline (0.9% NaCl) Adjust total volume based on patient tolerance and concentration
Infusion	Infuse over 30–60 minutes (slow, comfortable rate) Frequency: Weekly or as clinically indicated
Phases of Treatment	Initial: Weekly infusions for 4–6 weeks Maintenance: Every 2–4 weeks Longevity: Monthly for ongoing wellness and energy support
Monitoring & Precautions	Monitor: Serum electrolytes if frequent dosing Caution: Use carefully in patients with renal impairment (magnesium & calcium load) Observe for warmth or flushing during infusion; slow rate if needed
Adjuncts	⚠️ **Administer adjuncts after Myers' infusion. Do not mix or co-infuse in the same IV bag.** Common adjuncts include glutathione push, vitamin C, or NAD^+ to enhance energy metabolism, immunity, and cellular recovery.
Notes	Confirm compatibility of all ingredients prior to compounding Use sterile compounding and aseptic technique Label with formulation details, date, and initials

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NAD^+ IV Therapy

Section	Details
Dosage	Standard dose: 250–750 mg per infusion Therapeutic range: 250–1,000 mg depending on indication and protocol
Dilution	Dilute in 250–500 ml normal saline (0.9% NaCl) Concentration adjusted per patient tolerance and infusion duration
Infusion	Infuse slowly over 2–4 hours (higher doses may require 6+ hours) Frequency: 1–2 times weekly during initial phase
Phases of Treatment	Initial: 250–500 mg, 1–2× weekly for 4–6 weeks Maintenance: 500–750 mg every 1–4 weeks Longevity: 250–500 mg monthly for mitochondrial and anti-aging support
Monitoring & Precautions	Monitor: Heart rate, comfort level, and tolerance throughout infusion Rapid infusion may cause: Flushing, chest tightness, light-headedness, or anxiety Action: Slow or pause infusion if symptoms occur
Adjuncts	⚠️ **Administer after NAD^+ infusion. Do not mix or co-infuse with other nutrients in the same IV bag.** Common adjuncts include Myers' cocktail, B-complex, Vitamin C, or glutathione to enhance mitochondrial recovery and detoxification.
Notes	Encourage hydration before and after infusion Use light-protected tubing if prolonged infusion Label infusion with dose, volume, and date

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High Dose Vitamin C

Section	Details
Dosage	Starting: 25 g in 500 ml saline Max: 70 g in 1000 ml saline Range: 25–50 g, titrated upward
Dilution	500 mL NS for 25 g 1000 mL NS for 70 g Add: Magnesium chloride Add: Calcium chloride 10% at 3.5 mL per 25 g Vit C
Infusion	1.5–3 hours (slower at higher doses) Frequency: 1–3 times weekly
Phases of Treatment	Initial: 25 g in 500 mL, assess tolerance & labs Maintenance: 25–50 g, 1–2x weekly, 1–3 months Longevity: Monthly/as indicated for support
Monitoring & Precautions	Labs: G6PD, BUN, creatinine, electrolytes Contra: G6PD deficiency, renal issues, kidney stones Notes: Magnesium & calcium reduce burning/irritation
Adjuncts	⚠️ **Administer after Vitamin C IV Infusion. Do not mix or co-infuse with other nutrients in the same IV bag.** ⚠️ **Ozonated glycerin should NOT administered in the day of Vitamin C.** Common adjuncts include glutathione (post-infusion) Stack with B-complex, trace minerals, antioxidants
Notes	Encourage hydration before and after infusion Use light-protected tubing if prolonged infusion Label infusion with dose, volume, and date

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Ozonated Glycerin

Section	Details
Dosage	Using a 25 cc syringe, pour 7 cc of ozonated glycerin into an empty syringe (remove the plunger first, then reinsert it). Attach an 18-gauge needle and draw saline from the 250 mL bag. Mix the contents in the syringe until the solution becomes thin enough to return to the bag; this may require pushing and re-drawing several times. Finally, add 2 cc of DMSO and 6 cc of sodium bicarbonate.
Dilution	14 to 27 cc is 500 ml 28 and more is 1000 ml
Infusion	Infuse slowly over 2–4 hours (higher doses may require 6+ hours) Frequency: 1 time weekly on the day of NOT infusing Vitamin C
Phases of Treatment	1X week and increase ozonated glycerin weekly until max dose achieved
Monitoring & Precautions	Monitor: Heart rate, comfort level, and tolerance throughout infusion Rapid infusion may cause: Flushing, chest tightness, light-headedness, or anxiety Action: Slow or pause infusion if symptoms occur
Adjuncts	⚠️ **No other IV should be administered in the day of ozonated glycerin.**
Notes	**Must get hydration before and after infusion.**

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